

PRANAMS TO SUTTURU GURU PARAMPARA



WEL COME



UNDERSTANDING OF GARBHINI PARICHARYA AND IT'S PRACTICAL APPROACH



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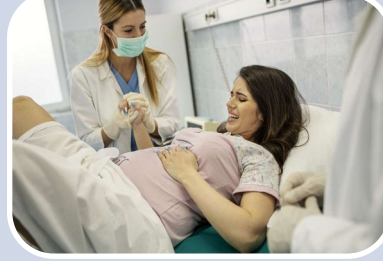
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INTRODUCTION



ऋतुमति

गर्भिणी

प्रसव

सूतिका

रजोनिवृत्ति

Ayurveda aims at –

स्वस्थस्य स्वास्थ्य रक्षणम्, आतुरस्य विकार प्रशमनम्.

दिनचर्या



रात्रीचर्या



ऋतुचर्या

स्वस्थस्य
स्वास्थ्य
रक्षणम्

DEFINITION

गर्भोऽस्त्यस्याम्

गर्भिणी – Pregnant women

Synonyms - आपन्नसत्त्वा,गुर्विणी,अन्तर्वत्नी.

परिचर्या – Care,service,activities.

Synonyms - वरिवस्या,शुश्रूषा,उपासना,पथ्य.



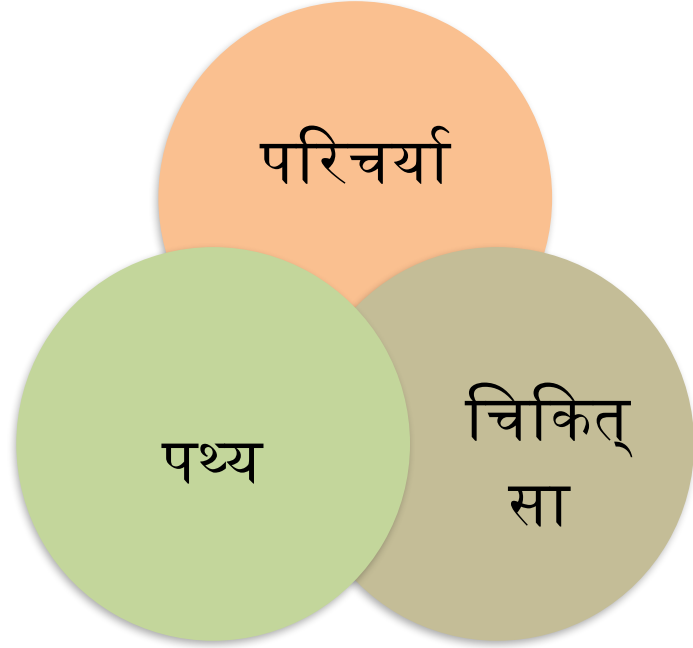
- ❖ **Pregnancy** is the most important phase in woman's life.
- ❖ It is a physiological condition requiring additional nutrition to fulfill the needs of mother and fetus.

पूर्णमिव तैलपात्रमसङ्क्षोभयताऽन्तर्वत्नी भवत्युपचर्या ।।
च.शा.8/22जातिसूत्रीयं अध्याय

सूक्ष्मां चिकित्सां वक्ष्यामि गर्भिणीनां विभागशः।

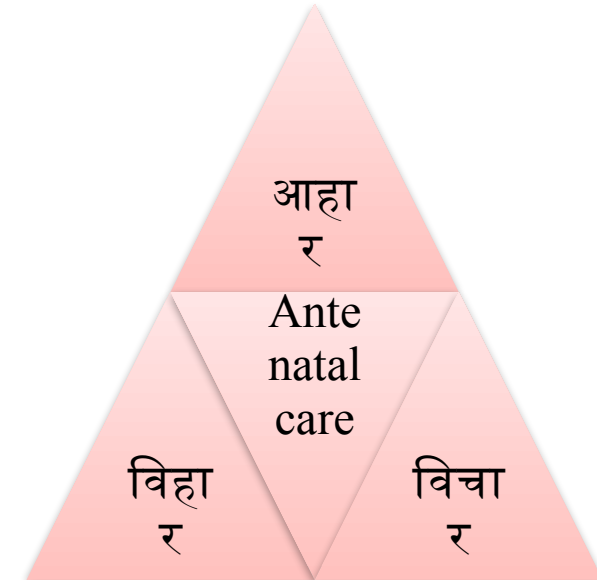
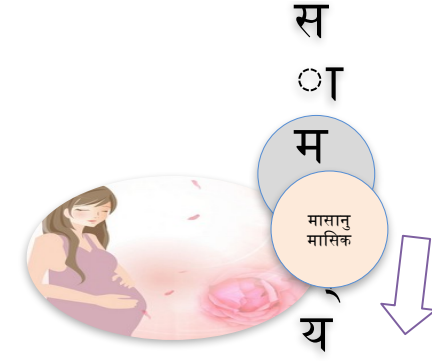
तथा गर्भश्च नारी च वर्धते रक्ष्यतेऽपि च ॥

का.खि.अन्तर्वत्नी चिकित्सितम् अध्याय



पथ्येसति गदार्तस्य किमौषध निषेवणैः।

पथ्ये असति गदार्तस्य किमौषधि निषेवणैः॥ वैद्य जीवन



ANTENATAL CARE

The aims

- (1) To screen the “high risk” cases.
- (2) To prevent or to detect and treat a complication.
- (3) To ensure continued risk assessment and to provide ongoing primary preventive health care.
- (4) To educate the mother about the physiology of pregnancy.
- (5) To discuss with the couple about the place, time and mode of delivery, provisionally and care of the newborn.
- (6) To motivate the couple about the need of family planning.



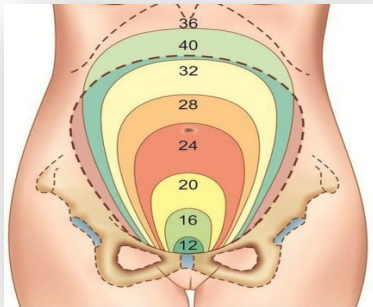
Systematic supervision of
a woman during pregnancy

Objective

Is to ensure a normal pregnancy with delivery of a healthy baby from a healthy mother.

ANTENATAL CARE COMPRISES OF

Clinical
Couplee
education
Advisal
labing



सामान्य गर्भिणी परिचर्या

मात्रादीनां खलु गर्भकराणां भावानां सम्पदस्तथा

आहार



विहार



VACCINATION

Indicated	Contraindicated	May be
➤ T T ➤ Inactivated influenza ➤ Anti D Immunoglobulin	MMR, Varicella	➤ Pneumococcal ➤ Meningococcal ➤ Hepatitis A, Hepatitis B

DIET

The diet during pregnancy should be adequate to provide:

- (a) Good maternal health,
- (b) Optimum fetal growth,
- (c) The strength and vitality required during labor and
- (d) Successful lactation

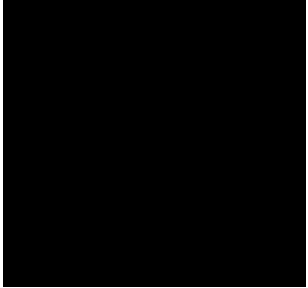
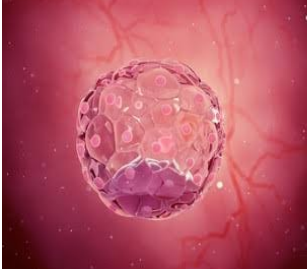
Dietary components	Nonpregnant	Pregnancy second half	Lactation	Sources
Energy (kcal)	2,200 kcal	2,500 kcal	2,600 kcal	Protein, fat, carbohydrate
Protein (g)	50 g	60 g	65 g	Meat, fish, poultry, dairy product
Iron (mg)	18 mg	40 mg*	30 mg*	Meat, egg, grains [* to be supplemented]
Calcium (mg)	500 mg	1,000 mg	1,500 mg	Dairy products
Zinc (mg)	12 mg	15 mg	19 mg	Meat, egg, seafood
Iodine (µg)	150 µg	175 µg	200 µg	Iodized salt, seafood
Vitamin A (IU)	5,000 IU	6,000 IU	8,000 IU	Vegetables, liver, fruits
Vitamin D (IU)	200 IU	400 IU	400 IU	Dairy products
Thiamine (mg)	1.1 mg	1.5 mg	Almost same	Grains, cereals
Riboflavin (mg)	1.1 mg	1.6 mg		Meat, liver, grains
Nicotinic acid (mg)	15 mg	17 mg		Meat, nuts, cereals
Ascorbic acid (mg)	60 mg	70 mg	as in pregnancy	Citrus fruits, tomato
Folic acid (µg)	200 µg	400 µg		Leafy vegetables, liver
Vitamin B₁₂ (µg)	2 µg	2.2 µg		Animal proteins

IRON AND CALCIUM SUPPLEMENTS



मासानुमासिक गर्भिणी परिचर्या

MONTH	आचार्य चरक	आचार्य सुश्रुत	अष्टांग संग्रह	हारित
1 ST	- अनुपसंस्कृत क्षीर - सात्त्विक भोजन	मधुर, शीत, द्रव आहार	- उपसंस्कृत क्षीर, क्षीरोद्धृत सर्पिं processed with शालपर्णी, पलाश क्वाथ with कनकरजत क्वथित शीतोदक - मधुर, शीतल, द्रव आहार - अभ्यंग, उद्धर्तन निषेध	मधुयष्टि, मधूकपुष्प, परुषक with नवनीत & मधु, मधुरद्रव्ययुक्त क्षीर
2 ND	मधुरौषधिसिद्ध क्षीर	मधुर, शीत, द्रव आहार	मधुरौषधि सिद्ध क्षीर	काकोलिसिद्ध मधुर क्षीर
3 RD	मधुघृत सिद्धक्षीर	- मधुर, शीत, द्रव आहार - षष्टिकशालि अन्न/ओदन with क्षीर	मधुरौषधि सिद्ध क्षीर	कृशरा



1st trimester

क्षीर

स्वादु शीतं मृदु स्निग्धं बहलं श्लक्ष्णपिच्छिलम्।
गुरु मन्दं प्रसन्नं च गव्यं दशगुणं पयः॥२१७॥
च.सू.27.अन्नपानविधि अध्याय

क्षीर-जीवनीय

Milk –

- Complete food
- Calcium, riboflavin, phosphorous, vitamin A, B12, potassium, magnesium, zinc, iodine.
- Prevents neural tube defects
- Stabilises pregnancy
- Prevents heart burn

मधुररस-

- सात्म्य, सप्तधातु, ओजोवर्धक, षडीन्द्रिय प्रसादन, प्रीणन, जीवन, तर्पण, बलवर्णकर, पित्तविषमारुतघ्न, तृष्णादाह प्रशमन, त्वच्य, केश्य, बल्यं, स्थैर्यकर
- **नवनीत**-संग्राहि, दीपनीय, हृद्य (च.सू.27.अन्नपानविधि अध्याय)

मधु-

- हृद्य, नवीन (सर, बृंहण, नातिश्लेष्महर) सु.सू.45.द्रवद्रव्य विज्ञानीय अध्याय

MONTH	आचार्य चरक	आचार्य सुश्रुत	अष्टांग संग्रह	हारित
4 TH	•क्षीरनवनीत -अक्षमात्र (2तोला)	•षष्टिकशालि ओदन with दधि •हृद्य अन्न with जांगल मांसरस/क्षीर/नवनीत	•अक्षमात्र नवनीतयुक्त क्षीर	•संस्कृत ओदन भेल •संस्कृत क्षीर with नवनीत
5 TH	•क्षीरसर्पि	•षष्टिकशालि ओदन with क्षीर •हृद्य अन्न with जांगल •मांसरस/क्षीर/सर्पि	•क्षीरसर्पि	•पायस भेल •क्षीरयवागू
6 TH	•मधुरौषध सिद्ध क्षीरसर्पि	•गोक्षुरसिद्ध घृत /यवागू	•मधुरौषधि सिद्ध क्षीरसर्पि	•मधुरदधि भेल •क्षीरसर्पि



2nd trimester

घृत-

घृतं पित्तानिलहरं रसशुक्रौजसां हितम्।
निर्वापणं मृदुकरं स्वरवर्णप्रसादनम्॥१४॥

च.सू.13.स्नेहाध्याय

घृत-स्मृति, बुद्धि, अग्नि, ओज, कफ, मेद वर्धकर

मधु-हृद्य, नवीन (सर, बृंहण, नातिश्लेष्महर) सु.सू.45.द्रवद्रव्य
विज्ञानीय अध्याय

मधुररस-

- सात्म्य, सप्तधातु, ओजोवर्धक, षडीन्द्रिय प्रसादन, प्रीणन, जीवन, तर्पण, बलवर्णकर, पित्तविषमारुतघ्न, तृष्णादाह प्रशमन, त्वच्य, केश्य, बल्यं, स्थैर्यकर
- दधि-स्निग्ध, कफमेदोविवर्धन, काशर्यापह
- दधिमस्तु-बल्य, प्रीणन, तृष्णाक्लमहर

Milk –

- Support normal growth and brain development.
- Promotes muscle growth
- Superior fetal growth in terms of fetal length and head circumference
- Conjugated linolenic acid, omega -3 fatty acids – reduces risk of diabetes and heart disease.

Honey –

- Prenatal vitamins, minerals, antioxidants beneficial for both maternal health and fetal development
- Polyphenols and other beneficial compounds reduce the risk of complications.

Butter –

- Vit A,D,E – healthy eyes,strong bones,healthy skin.

Curd –

- Probiotics - Improves digestion,relieves constipation, reduces risk of gestational diabetes
- Boosts immunity
- Calcium,VitD,B12,potassium,magnesium – vital for bone health,nerve function,overall well being
- Potassium - Reduces risk of developing high blood pressure
- Lactic acid – Good for skin and hair

Ghee –

- Cholesterol rich saturated fat that forms building blocks of progesterone
- Omega 3,6,9 fatty acids – Foetal brain development,overall health,relieves constipation,improves digestion,nourish baby.
- Hormonal balance.
- Vit A,D,E,K – Support immune function,bone health,and skin health.
- Butyric acid – Anti inflammatory reduce inflammation in the body,including inflammation in the uterus during labour.

MONTH	आचार्य चरक	आचार्य सुश्रुत	अष्टांग संग्रह	हारित
7 TH	•मधुरौषध सिद्ध क्षीरसर्पि	•पृथक्पण्यादि/ विदारिगन्धादि सिद्ध घृत	•मधुरौषधि सिद्ध क्षीरसर्पि	•घृतमण्ड
8 TH	•घृतमिश्रित यवागू  	•आस्थापन बस्ति - बला, अतिबला, शतपुष्पा, पलल, दुग्ध, दधि, दधिमस्तु, तैल, लवण, मदनफल, मधु, घृत, बद र क्वाथः अनुवासन बस्ति - क्षीर, मधुरौषधी क्वाथसिद्ध तैल. •स्निग्ध यवागू, जाङ्गल मांसरस	•दुग्ध/घृतमिश्रित यवागू Rest same as सुश्रुत अष्टांगहृदय •घृतयुक्त क्षीरपेया, पुराणमल शुद्धि - मधुरवर्ग औषधि सिद्ध घृत •निरुह बस्ति - शुष्कमूलक, बदर, आम्लपदार्थ, शतपुष्पाकल्क, घृत, तै ल, सैन्धव. मधुरौषध सिद्ध तैल - अनुवासन बस्ति, पिचु (गर्भमार्ग, आशय स्नेहनार्थ)	•घृतपूरक  
9 TH	•मधुरौषध सिद्ध तैल - अनुवासन, पिचु 	-----  	•मधुरौषध सिद्ध तैल - अनुवासन बस्ति, पिचु (गर्भमार्ग, आशय स्नेहनार्थ) अष्टांग हृदय •स्नेहयुक्त मांसरस •स्नेहयुक्त यवागू •अनुवासन बस्ति and पिचु, वातघ्न द्रव्य पत्र शीतल क्वाथ स्नान	•9 th and 10 th month अन्नप्रयोग भेल •अनुवासन बस्ति - कदम्बमाष तैल, यवागुपान

3rd trimester

7th month

विदारीगन्धादि घृत Helps for growth and development of fetus and for fetal lung maturity.

8th month

आस्थापन बस्ति-

- Relieve constipation, gives strength to garbhashaya and prevent preterm labour by preventing udavarta and akala avi pradurbhava.
- Asthapana basti is claimed to have rasayana properties so it may help in normalizing anatomy and physiology of reproductive organs for preparation of parturition.

9th month

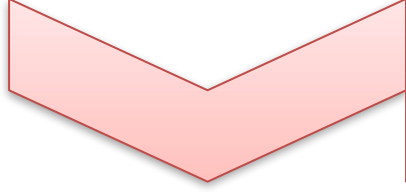
अनुवासन बस्ति -

- Vatanulomana, margasnehana

योनिपिचु-

- Lubrication of cervix, vaginal canal, perineum.
- Cervical remodeling
- Facilitates easy delivery
- Increases stretching capacity of vagina
- Prevents laceration of perineum

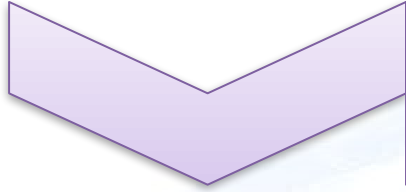
मासानुमासिक पथ्य लाभ



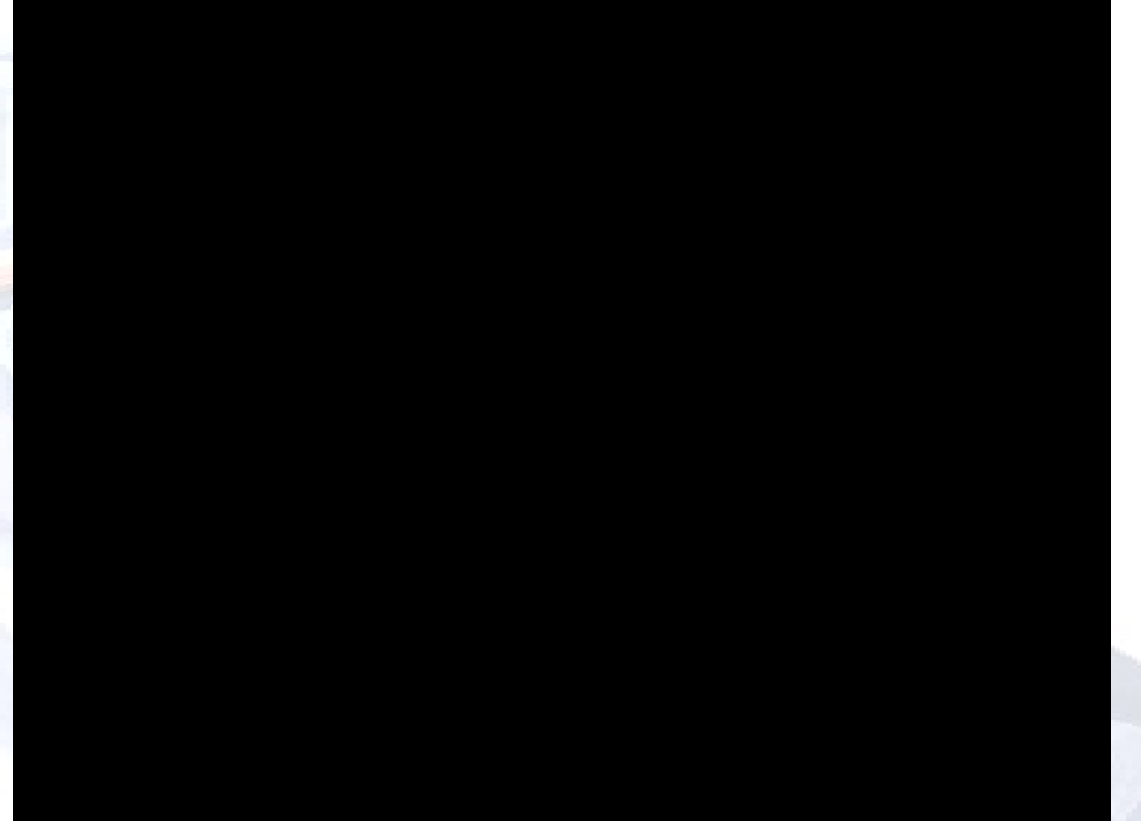
- कुक्षि,कटी,पार्श्व,पृष्ठ मृदुता
- वातानुलोमन



- मूत्र,पुरीष प्रवृत्ति
- चर्म,नख मार्दवता



- बलवर्ण उपचय
- सुखप्रसव



गर्भोपघातकर भाव



- ,अति उष्ण, अतिरुक्ष अन्न पान अति कटु, तिक्त भोजन
- गुरु, विष्टम्भि भोजन,
- लशुन प्रयोग

आहार



- दिवास्वप्न
- रात्रि जागरण,
- रक्तवस्त्रधारण
- वेगविधारण
- भारवहन, गुरुप्रावरण
- अभिघात
- उत्तानशयन

विहार



- क्रोध
- शोक
- भय
- उद्वेग

मानसिक

वर्ज्य आहार विहार सेवन परिणाम

उत्कट, विषम, कठिणासन, वात मूत्र पुरीष वेगाधारण, दारुण, अनुचित व्यायाम, तीक्ष्ण अतिमात्र सेवन, प्रमिताशन

अन्तर्मृत गर्भ, अकाल प्रसव, गर्भस्राव, गर्भाशोष

अभिघात, प्रपीडन, श्वभ्र कूपप्रपात, देशावलोकन, अभीक्ष्ण संक्षोभियान, अप्रिय, अतिमात्रश्रवण

अकाल प्रसव, गर्भस्राव

प्रततोत्तान, विवृत शयन

Umbilical cord encircles the neck of the fetus and troubles it

नक्तचारिण

उन्मत्त

व्यवायशील

Deformed, lazy

अभिद्यात्री

परोपतापित

कलीकलहशील

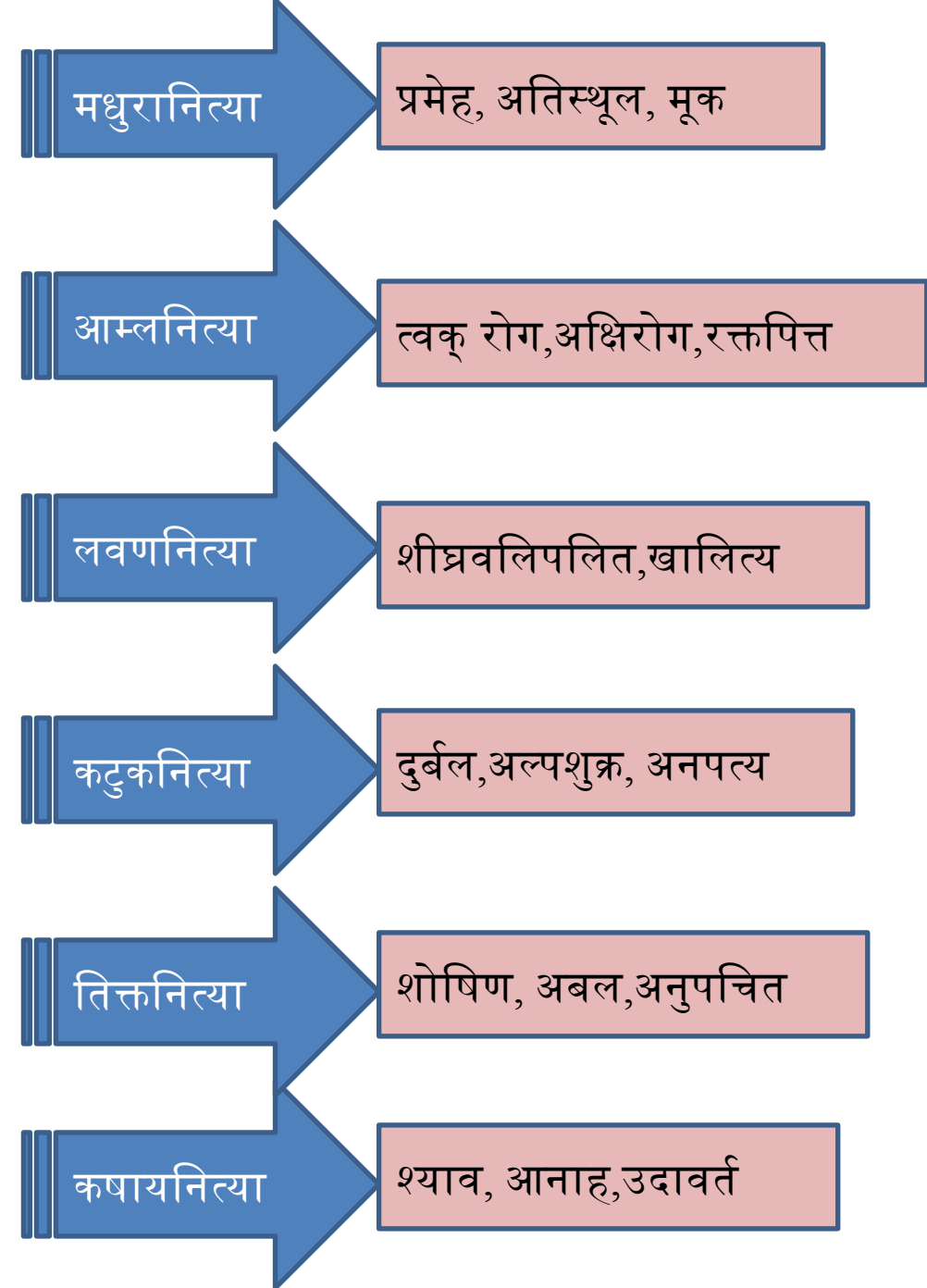
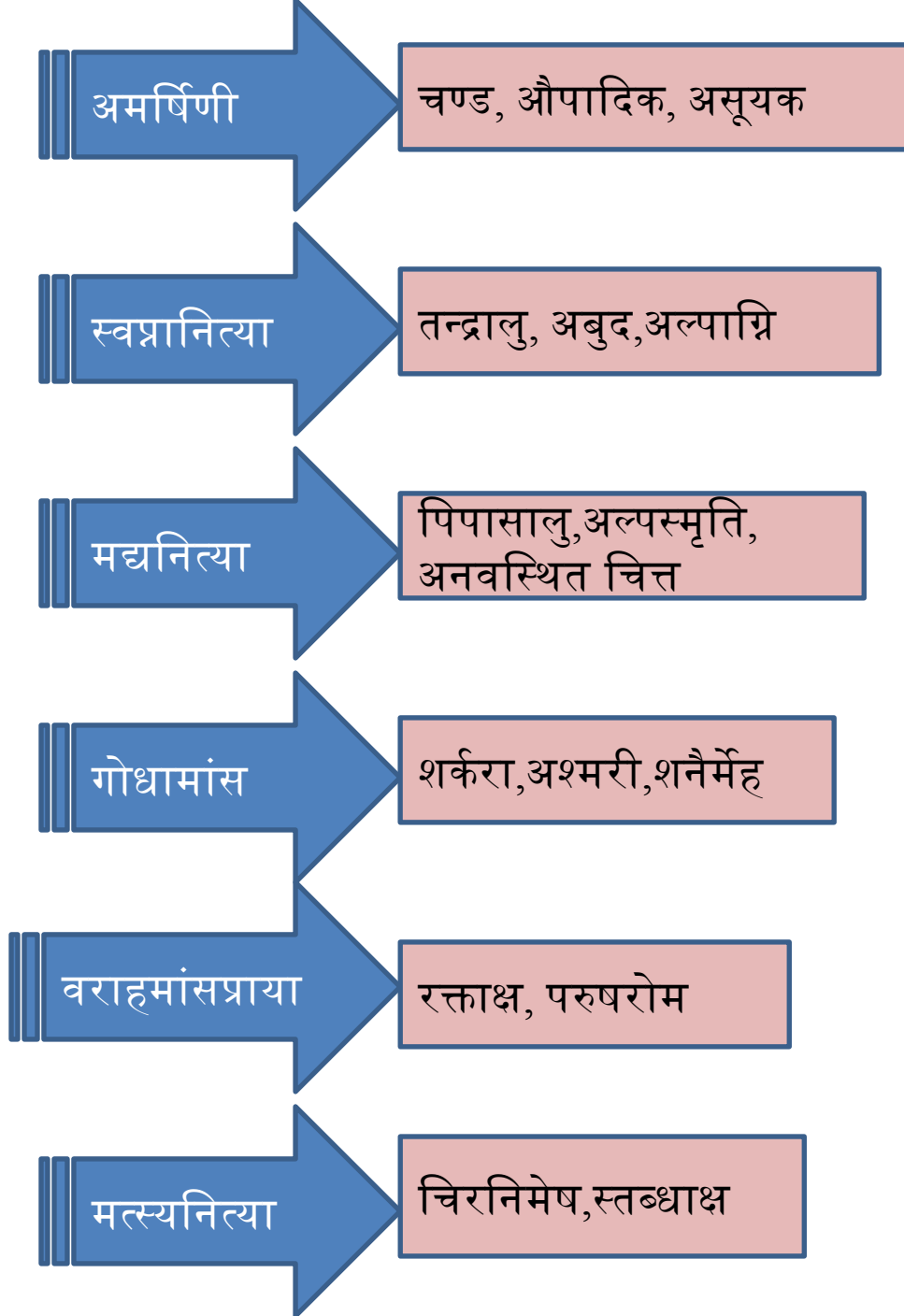
अपस्मार

शोकानित्य

भीत, अपचित,
अल्पायुष

स्तेन

आयासबहुल, द्रोहिण,
अकर्मशील



गर्भस्थापन औषधि

-

सम्प्रति स्थितस्य गर्भस्य गर्भोपघातकभाव प्रभाव खण्डकत्वेन
यत् पुनः स्थितिकारकं तद्गर्भस्थापनमुच्यते | च.शा.8/20 चक्रपाणि ठीका

□ Garbhashthapana dravyas are those which after counteracting the effect garbhopaghatakara bhavas helps in proper nourishment, growth and development of the foetus.

गर्भस्थापनानियैर्गर्भो न स्रवति | अ.सं.शा.1/62 इन्दु ठीका

Garbhashthapana drugs prevents abortion.

प्रजास्थापन महाकषाय



जीवनीय महाकषाय



प्रजास्थापन द्रव्य



ऐन्द्री
ब्राह्मी
शतवीर्या
सहस्रवीर्या
ऽमोघा
ऽव्यथा
शिवा
ऽरिष्टा
वाट्यपुष्पी
विष्वक्सेनकान्ता

जीवक
ऋषभक
मेदा
महामेदा
काकोली
क्षीरकाकोली
मुद्गपर्णी
माषपर्णी
जीवन्ती
मधुक

कमल
कुमुद
कशेरुक
शृंगाटक
पुत्रञ्जीवक
न्यग्रोधादि
संधानीय
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त्रिकोनासन



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योगनिद्रा



GARBHINI PARICHARYA: ROUTINE CARE FOR PREGNANT WOMEN THROUGH AYURVEDA

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Masanumasika garbhavruddhi, masanumasika paricharya, yoga, pranayama, Garbopaghatakara bhava, garbha sthapana dravyas explained.

ABSTRACT

Care during pregnancy is a very important aspect of life. Ayurveda classics refers to care with (Garbhini Paricharya) & Yoga and Garbopaghatakara bhava (Substances which are harmful to the fetus). Garbhini Paricharya (Antenatal care) is a very beneficial for maintenance of pregnancy. Attempt has been made to understand the physiological significance of Garbhini Paricharya in detail.

Keywords: Antenatal care, Garbhasthapaka Dravya, Garbhini Paricharya, Garbopaghatakara Bhava, Masanumasika Paricharya, Yoga.

INTRODUCTION:



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GARBHINI PARICHARYA (ANTE-NATAL CARE) – VIEW OF HARITA SAMHITA (A TREATISE OF 10th CENTURY)

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Aahara, Garbhini Paricharya, Harita Samhita

ABSTRACT

Woman, being the precious creation of the world is the only source for new creation. To procreate a healthy progeny is one of the esteem desire of a woman. Pregnancy although being a physiological phenomenon, still it may be at risk to face any complication at any stage due to the altered anatomical and physiological functions of the body of a pregnant female. To assure a safe motherhood as well as a healthy progeny in her womb, special care of a pregnant woman is required which is elucidated as antenatal care. In Ayurvedic classics,

Early and month wise development of fetus, month wise diet and other procedures and analysis of benefits of regimen as per acharya harita explained in detail.

Antenatal care (ante-natal care) is a very important aspect of life. Harita Samhita suggests that he has charted out this scheduled diet pattern in a chronological order i. e. liquid to solid diet in order to prevent any complication arise during pregnancy and to provide a milieu interior to the body of mother as well as baby for better adapting power with the changing physiological phenomenon. Findings of this review article may validate the scientific validation of views of Harita Samhita recommended diet and habit for pregnancy.

BHESHAJA PRAYOGA IN GARBHASTHAPANA

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ABSTRACT

Garbhasthapana dravyas are those which after counteracting the effect of harmful factors that is *garbhopaghatakara bhavas* for foetus help in its proper maintainence. According to ashtanga sangraha indu theeka it is said that use of *garbhasthapana dravyas* prevents abortion. Acharya kashyapa also advised that the use of *garbhasthapana dravyas* for maintenance of proper health, growth and development of mother and foetus. *Ksheera* or *ghrita* medicated with drugs mentioned as *prajasthapana* and *jeevaneeya* in *shadvirechana shatashriteeya adhyaya* of charaka samhita sutrasthana are beneficial. The bath with cold decoction of these drugs during *pushya nakshtra* said to be beneficial. Amulet of these drugs tied in head or right arm. Acharya kashyapa also enumerated list of *garbhasthapaka dravyas* in kashayapa samhita khilasthana 10th chapter, these drus should be uprooted with enchantation of mantras and tied as amulet during

nushva nakshtra. The use of amulet of *trivrit* tied in waist of pregnant women also beneficial.

Garbhasthapana dravyas, garbhsthapana krama, ksheerapaka described

Pumsavana karma helps in conception, implantation and also prevents abortion. Other than *ksheera* and *ghrita* preparations of single and combination of drugs *yogas* like *phalaghrita*, *dhatryadi ghrita*, *kasherukadi ksheera paka*, *kushadi ksheerapaka*, *kashervadi churna* some patent medicines are also helpful for the prevention of abortion.

KEYWORDS: *Garbhasthapana, prajasthapana, garbhasrava, garbhachalana, pumsavana*

REVIEW ARTICLE

YOGA IN PREGNANCY: A BOON TO MOTHERHOOD

JYOTSNA THAKUR¹ EENA SHARMA² SAMEET MASAND³

ABSTRACT

Yoga in pregnancy is multi dimensional physical, mental, emotional and intellectual preparation to answer the challenges faced by a pregnant woman. The challenges of pregnancy are revealed by the state of happiness and stress while *yoga* is a skill to calm down the mind. Pregnancy in a woman is a condition in which woman changes both from inside as well as outside. These changes create obstacles or hurdles in the normal life of a pregnant woman and *yoga* in pregnancy can help the women to cruise through these changes and challenges. Practicing *yoga* during pregnancy provides a great range of activity and benefits to unborn child and mother by numerous ways. *Yoga* soothes the mind, refocus the energy and prepare the woman physiologically and psychologically for labour. Different breathing techniques impart invaluable neuro-muscular control and helps in coordinated relaxation and contraction of uterus. Different type of *asanas* (postures), are described in *Ayurveda* and *Yoga darshan* texts which can be performed by a pregnant woman as they consume low energy and provide greater benefits. Published articles and different studies with references have been considered to support the effect of *yoga* in pregnancy. *Yoga* practicing includes physical postures and breathing techniques which minimizes the complication of pregnancy, like pregnancy induced hypertension, intrauterine growth retardation and pre-term delivery etc. Western exercises bring about what is known as phase contraction of muscles while yogic exercises create a static contraction which maintains a muscle under tension without causing repeated motions^[1]. An approach to *yoga* in pregnancy can improve birth weight; decrease pre-term labour, decreased IUGR with least or no complications.

KEY WORDS – Pregnancy, Yoga, Asanas, Pranayama, Birth weight, stress

Trimester wise yogasanas its benefits, pranayama and mudras are explained

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GARBHASANSKAR – A CRITICAL REVIEW

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ABSTRACT

Together, Garbha and Sanskar mean shaping and cultivating the unborn while also reforming, polishing, and completing them. 'Garbha Sanskar' is the process of choosing and disseminating beneficial influences through yoga, reading virtuous scriptures, positive thinking, prayer, healthy diet, and joyful behaviour. Your link with your child begins the moment you become pregnant, despite the fact that this may seem bizarre and weird. It's not like you already know the baby when he's born. Even as it is growing inside of you, the baby listens to you and is sensitive to your emotions. By meditating, rubbing your infant gently, listening to calming music, and of course thinking positively, you can influence your baby's early impressions.

KEY WORDS: shreyasipraja, Pinda Shuddhi, designer baby.

INTRODUCTION

Ayurveda's "Shreyasipraja" can be obtained through Garbha Sanskar [1]. The idea of "Shreyasipraja" was mentioned by Acharya Charaka. Garbha Sanskar is a comprehensive procedure for maternal care, covering everything from conception planning to healthy baby delivery. According to Acharya Charak, the psyche of the parents, particularly the mother during pregnancy, is tied to that of the foetus.

Sanskar, which is in Sanskrit, denotes imparting excellent values or teaching good things. Garbha Sanskar then refers to educate an unborn child.

"Only after the union of Shukra and Shonit with Aatma in the uterus is the term Garbha. Only the union of shukra and shonit is not regarded as Garbha without the entry of Aatma.

"Sanskaro hi gunaantaradhanam".[2]

Sanskar means to replace undesirable traits with desirable ones. We can understand this simply as making modifications.

During pregnancy if efforts (Garbha Sanskar) are made by parents especially by mother then good traits will be inculcated in the foetus and may result in the birth of a baby with desired attributes. The method of Garbha Sanskar involves nursing the unborn child. During pregnancy, the mother's intellect and the unborn child are both being trained. The practise of a few straight forward exercises known as "Garbha Sanskar" promotes the social, physical, spiritual, and mental health of both mother and child. Scientific research has made

NEED OF GARBHA SANSKAR

- To suppress
- Increased
- More nutritious

In Ayurveda the concept of Pinda Shuddhi or detoxification of the body is very important. If the mother is not healthy, it may have a direct impact on child's health.

GARBHA SHAN

A farmer requires four things in order to cultivate a healthy plant: a good or ideal season to plant the seed, a healthy seed, fertile soil, and ideal water. Similar analogies are used by Ayurveda to describe the birth of a healthy child.[3].

1) Rutu - Proper period or time for conception. (Raja Samaya)

2) Kshetra - a mother or woman with a healthy uterus. (Garbhashaya)

"Mantras in pregnancy- month wise"

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ABSTRACT:

In today world different forms of *Supraja sanskar, garbha sanskar* , some thing additional to the regular ANC care have become very popular amongst the People as well as Healthcare providers .Along with *Yoga* , Diet and medicines *Mantra* and music are an integral part of all these programmes. *Mantra-shakti* is proven to be very powerful for pregnant women even in this materialistic world. It creates a high vibration, a sacred sound, and a deep, loving peace for yourself and your growing baby. Chanting mantras can uplift your soul and you may feel a close connection to specific *mantras* during pregnancy. This process is also known as '*Garbha Sanskar*' as per the ancient tradition. In *Garbhini* texts have advised *mangal swasti vachan* for the *garbhini* to create positivity and ward off evil forces. *Mantras* generate positive energy and all the mantras have

specific purposes. Chanting mantras provide peace of mind and purity at heart. Mantra is a powerful tool for peace and joy during these days of tremendous changes of pregnancy and motherhood and you must chant them throughout your pregnancy for an easy delivery and a healthy you and a healthy baby. Month wise *mantras* have also been described in our ancient texts. For eg. During the first month the fertilized ovum is in the liquid state and this process takes place through intercourse for the purpose of the birth of life this month is ruled by planet Venus. Hence *Om Shukray Namah mantra* should be chanted.

Use of *mantra* chanting has

Monthwise mantras are described

article is to describe the different *mantras* and their effects in pregnancy. using *mantra* in pregnancy and in birth can



**A COMPARATIVE CLINICAL STUDY ON THE EFFECT OF
SATAVARI MANDURA AND AMALAKI AVALEHA IN THE
MANAGEMENT OF PANDU IN GARBHINI W.S.R TO ANAEMIA IN
PREGNANCY**

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ABSTRACT

India is one of the countries with high prevalence of anaemia during pregnancy. Anaemia is multifactorial. Iron deficiency anaemia is the most common conditions in a pregnant woman. As per *ayurvedic classics*, this condition occurs due to improper *Rasa Dhatu* in mother and continuously increasing fetal demands and is considered as *Rasa Pradoshajavikara*. A large number of *Lauha* preparations have been used widely for centuries to cure Anaemia.

Aim: To evaluate efficacy of *Shatavari Mandura* and *Amalaki Avaleha* on *Garbhini Pandu*.

Materials and Methods: A total 30 pregnant women with symptoms of *Garbhini Pandu* were randomly divided into three groups (A, B and C). In Group A *Satavari mandura*, 3gms in divided dose for the period of 30 days. In Group B *Amalaki avaleha*, 12 gms in divided dose for the period of 30 days. In Group C Ferrous sulphate in the dose of 200mg (containing 60mg of elemental iron) was administered once daily for the period of 30 days. Parameters of signs and symptoms & investigations was scored on the basis of standard method and was analysed statistically.

Results: The results revealed that overall clinical improvement was better in Group A when compared to Group B&C. Hemoglobin was increased in patients of Group A, which was statistically significant. No adverse drug reaction was observed during the treatment period.

Conclusion:

Shatavari Mandura is more effective on *Garbhini Pandu* in comparison to *Amalaki Avaleha*.

Keywords: Anaemia, *Shatavari Mandura*, *Garbhini Pandu*, *Amalaki Avaleha*.

INTRODUCTION: Pregnancy is a condition itself considered as precious. In this situation Pregnant women treated as very special. Diseases during pregnancy considered serious issue and needs immediate response, otherwise it gives adverse effect on both mother and fetus. where it has been estimated that 41.8% of pregnant women worldwide are anaemic¹. The majority (at least half) of this burden is due to iron deficiency². However, there is a significant variation in prevalence of anaemia,

both within and between countries. Because of physiological changes during pregnancy, pregnant women are at higher risk of anaemia and in particular iron deficiency anaemia, which is the most common type of anaemia during pregnancy. Hematological changes during pregnancy, especially expansion of blood volume, often confuse the diagnosis of anaemia and its treatment. Because of increased iron and folic acid demands during pregnancy, pregnant women are more susceptible to

**A COMPARATIVE CLINICAL STUDY ON THE EFFECT OF
SATAVARI MANDURA AND AMALAKI AVALEHA IN THE
MANAGEMENT OF PANDU IN GARBHINI W.S.R TO ANAEMIA
IN PREGNANCY**

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Conclusion: *Shatavari Mandura* is more effective on *Garbhini Pandu* in comparison to *Amalaki Avaleha*.



A patient aged 25yrs ML-5Yrs,P1L1,approached for treatment for the 2nd child concieved after 1year of treatment diagnosis – delayed menstruation,confirmed with UPT

8/1/24

USG

SLIUP – 6wks

Yolk sac seen

Treatment

Adviced diet concentrating on liquid and semisolids

Tab. Folsafe 5mg 0-0-1

syp. Zymnet plus 10ml -0 -10ml

17/2/24

Blood group,Rh typing – O positive

BT – 3’30’’

CT -5’

RBS – 81mg/dl

TSH – 0.03miu/ml

Urine routine – NAD

Result

17/2/24

NT Scan

SLIUP – 12w+2d

NT – 1.4mm

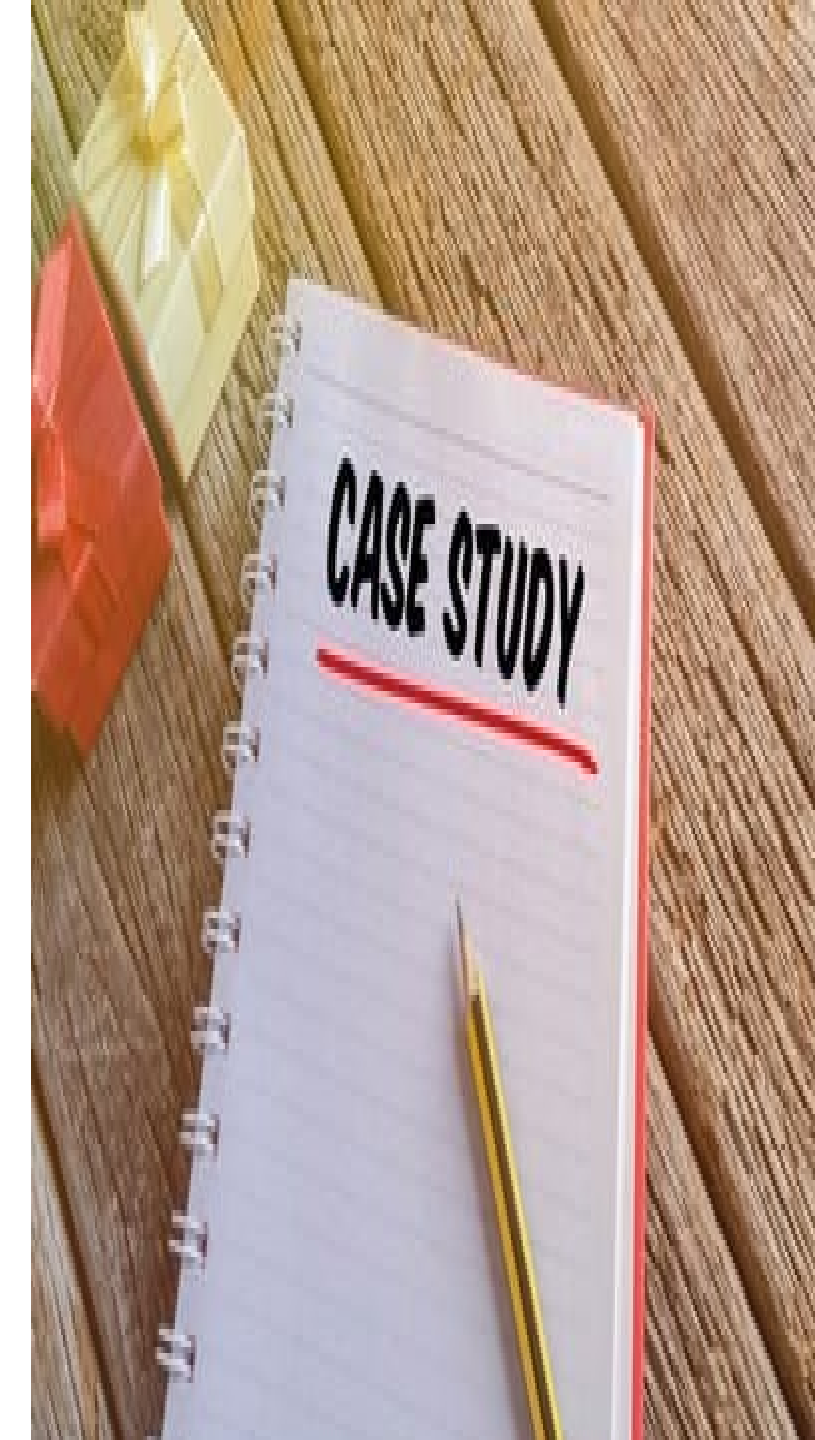
NB – Seen

Liquor – Adequate

Placenta – Fundo body posterior

Uterine artery doppler – Negative for PIH

Cervacal length - 3cm,Int OS closed



CASE STUDY

A patient aged 31 yrs, ML - 7yrs, G2P1L1, 1
1/2 month amenorrhoea

UPT –Positive
Advised USG

USG (4/8/23)
SLIUP – 8w+3d

USG 4/9/23
SLIUP – 12w+6d
NT – 1.7mm
NB –Seen
CL – 3.3cm, Int OS closed.

Hb% - 11.3m/dl

Inj TT – 1st – 5/8/23
2nd – 4/9/23

1st trimester

- Diet advised
- Madiphala rasayana 10ml – 0
10ml BF
- Folsafe 5mg 0-0-1
- Phalaghrita 0-0-5ml

2nd trimester

- Punarnava mandoora 1-0-1
- Amyron syp 10 ml – 10ml- 10ml
- Reosto cap 1-0-1
- Sukumara ghrita 0-0- 5ml

Result

3/11/23

SLIUP – 21w+3d

CL -3.9cm

Liquor – Adequate

Placenta – Fundal, anterior, towards
right side.

No gross anatomical defect



A patient, 21 yrs, ML -
1yr, Nullipara, approached for infertility
treatment. conceived after 1yr 3months of
treatment

1st TT – 31/7/23

2nd TT – 2/9/2023

24/7/23

SLIUP -7w+4d

Yolk sac seen

24/8/23

SLIUP – 12w+3d

NT – 1.1mm

NB – Seen

Liquor – Adequate

Placenta – Fundo body anterior

Uterine artery doppler – Negative for
PIH

Cervical length - 3cm, Int OS closed

31/10/23

SLIUP – 21w+5d

Liquor – Adequate

Placenta – Fundo body anterior

Uterine artery doppler – Negative for PIH

Cervical length - 3cm, Int OS closed

9/1/24

SLIUP 31w+5d with cephalic presentation.

Doppler – Normal

Liquor – Adequate

Placenta – Fundobody posterior, G -2

EFW – 1786gms±261gms

Hb% - 11.4gm/dl

FBS – 76mg/dl

PPBS – 91m/dl

HIV - Negative

HbsAg – Negative

Blood grouping and Rh typing – B Positive

TSH – 1.314mIU/ml

1st trimester

Syp Amyron 10ml-10ml-10ml

Tab.Folsafe 5m 0-0-1

2nd trimester

Tab.Dhatriloha 1-0-1

Cap .Reosto 1-0-1

3rd trimester

Tab. Amyron 1-1-1

Cap .Reosto 1-0-1

Cap.KB 101

Shatavarex granules

Result –

FTND – 26/2/24 - Sex -Female - Weight - 2.8
kg





A patient, 23yrs, ML -
2yrs, Nullipara approached for
infertility treatment, conceived
after 1yr 3months of treatment.

Hb % - 8.7gm/dl
RBS - 124mg/dl
Urine routine - NAD

7/11/22
SLIUP-8wks

5/12/22
SLIUP -11w+3d
Low lying placenta
NT - Normal
NB -seen

3/4/23

SLIUP - 29w+5d with breech
presentation
Liquor - Adequate
Placenta - Fundo body anterior
Doppler - Normal
EFW - 1386gms $\pm$ 203gms

27/5/23

SLIUP 37w+4d with cephalic
presentation.
Liquor - Adequate
Placenta - Fundo body anterior
Doppler - Normal
EFW - 3167gms $\pm$ 462gms

1st trimester

Syp Zymnet plus 10ml-10ml-10ml

Tab.Folsafe 5m 0-0-1

2nd trimester

Tab.Punarnava Mandoora 1-0-1

Cap .Me - cal 1-0-1

Dadimadi ghrita 0-0-5ml

3rd trimester

Punarnava Mandoora 1-0-1

Cap .Me - cal 1-0-1

Limiron granules 1tsp with milk

Result - LSCS – 2/6/2023 – 5:20PM -
Female – 2.8kg

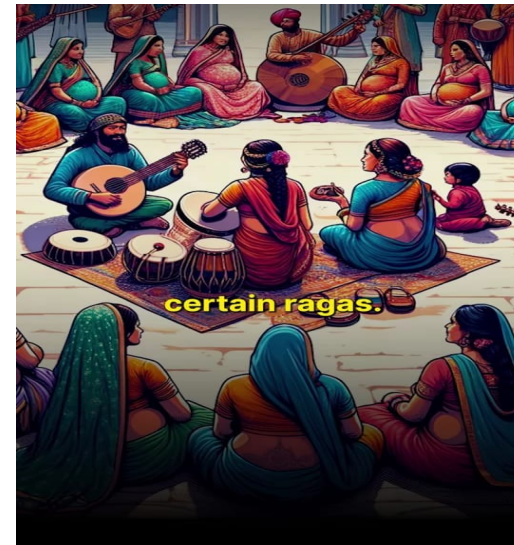
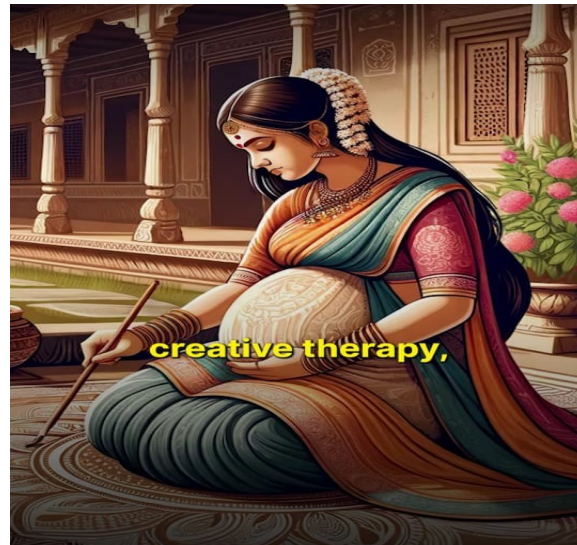
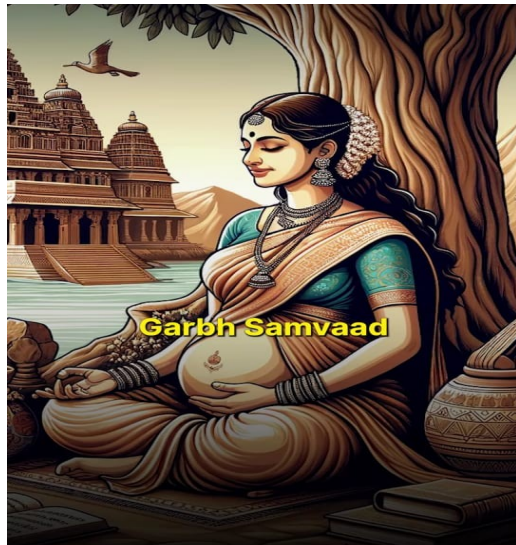


Pause, think, act.

- Many complications in obstetrics often arise as emergency and without any warning during pregnancy, labor and puerperium. These are hemorrhage (APH, PPH), hypertension (eclampsia), premature rupture of membranes, unexplained intrauterine fetal death, cord prolapse.
- These are the important causes of maternal morbidity and mortality in India. Simultaneous availability of emergency obstetric care (EmOC) should be there to combat these complications.
- Therefore good antenatal care and efficient EmOC are complementary to each other for successful obstetric outcome.

INNOVATIVE PRACTICES

❑ Garbha samskara





Narikela rasayana



Udaralepa

DISCUSSION



➤ 1st month गर्भ is in the form of कलल Pregnant women experiences symptoms like nausea ,vomiting,tiredness etc.आचार्या 's explained उपसंस्कृत क्षीर or अनुपसंस्कृत क्षीर, मधुर,शीत,द्रव आहार. यष्टिमधु acts as anti emetic परुषक is rich in iron ,calcium,sodium, potassium,and vitamin C. अनुपसंस्कृत क्षीर is given to relieve vomiting,dehydration.

➤ 2nd month garbha starts to take compact form.So आचार्या,s have adviced to take मधुरौषधि सिद्ध क्षीर,काकोलीसिद्ध क्षीर,मधुर शीत द्रव आहार. काकोली is जीवनीय द्रव्य acts as गर्भस्थापन

➤ 3rd month body parts starts to differentiate.It helps for growth and development of neural tissue. मधु घृत सिद्ध क्षीर, षष्टिक शालि ओदन, कृशरा helps in the formation of body parts.

➤ 4th month more stability and differentiation at cellular,tissue and organ level.Expression of चेतना धातु becomes more prominent Which indicates functioning of heart in fetus.नवनीत - वर्धन,मासरस - प्रीणन,हृद्य, षष्टिक शालि – स्थिरत्व.

➤ 5th month there मांसशोणीतोपचय, enlightening of मनस् and fetus becomes सुजीव, hence मांसरस is administered. त्वचा is उपधातु of मांसधातु - घृत is त्वच्य and enhances the properties of त्वचा in fetus. क्षीर is मनस्कर helps for proper functioning of manas. In the 5th month due to मांस शोणीतोपचय pregnant women experience काश्यता hence क्षीर, सर्पि, मांसरस, पायस, षष्टिकशालि ओदन adviced.



➤ 6th month there is more enlightenment of बुद्धि, बलवर्णोपचय, केश, लोम, नख, अस्थि, सिरा, स्नायु formation pregnant woman experience बलवर्ण हानि, अधिक श्रम and due to increased pressure on lower limbs and water retention there is oedema of legs, hence she is adviced to take क्षीरसर्पि, मधुरौषध सिद्ध क्षीरसर्पि, मधुर दधि, गोक्षुरसिद्ध घृत or यवागू.

7th month all the body parts of fetus are fully develops, becomes more conspicuous and associated with त्रिदोष's. Adviced to take मधुरौषध सिद्ध क्षीरसर्पि, घृतमण्ड, पृथकपर्ण्यादि or विदारीगन्धादि घृत. Helps for growth and development of fetus and for fetal lung maturity.



8th month - As ओजस् becomes unstable घृतपूरक, आस्थापन बस्ति advised, which stabilizes the ओजस् and strengthen the immune system of the fetus. अनुवासन and आस्थापन बस्ति helps for पुराणमल शोधन and वातानुलोमन.

9th month आस्थापन and अनुवासन बस्ति helps for वातानुलोमन, पुराणमल शोधन and सुख and निरुपद्रव प्रसव, पिचु helps for स्नेहन of मार्ग.

Yoga is an ideal way for physical, mental, social, spiritual well-being. Yoga can be effectively advocated during all three trimesters of pregnancy under expert guidance. During pregnancy practice of yoga helps in controlling emotions and mood swings resulting due to hormonal changes.

CONCLUSION

- In present days women follow certain practices that are contraindicated during pregnancy and tend to ignore the advices of our ancestors, leading to some obstetrical complications. We have to find a bridge between our tradition, ancestral wisdom and practicality.
- Ayurveda states the importance of caring for the mother before, during and after pregnancy. It talks about do's and don't's, which are garbhashthapaka bhavas and garbhapaghatakara bhavas. Ahara, vihara and vichara are tools for the management of pregnancy and it plays a significant role in the health of pregnant women and development of the foetus.
- ANC - Regular and periodic examination, investigations, advice helps to screen high risk cases and also helps in prevention or detection and treatment of any complications at the earliest.

REFERENCES

1. Agnivesha, *Charakasamhita*, revised by Charaka and Drudabala, *Ayurveda Deepika of Chakrapanidatta*, edited by Vaidya Jadavji Trikamji Acharya, *Shareerasthana*, Chapter Number 8, *Jatisutriya shareera*, New Delhi, *Chaukhambha publications*, 344.
2. Vrddhajivaka, *Kashyapa samhita*, revised by Vatya with Sanskrit introduction by Nepal Rajguru Pandit Hemaraja Sharma with Vidyotini Hindi commentary and Hindi translation of Sanskrit Introduction by Ayurvedalankara Sri Satyapaala Bhishagacharya, *Khilasthana*, Chapter Number 10, *Antarvartni Chikitsa Adhyaya*, Bhavan, Varanasi.
3. Dutta. D C, Edited by Kona,,r Hiralal, Textbook of Gynecology including contraception, 7th edition, Chapter Number 10, Antenatal care and preconceptional care , Delhi, Chaukhamba Publications, Reprint: 2022; p 106 - 115.
4. Sushrutacharya, *Sushruta samhita with Nibandhasangraha commentary of Sri Dalhanacharya and Nyayachandrika Panjika of Gayadasacharya on Nidanasthana*, Edited by Vaidya Jadavaji Trikamji Acharya, *Shareerasthana*, Chapter Number 10 *Garbhini vyakarana Shareera adhyaya*, Varanasi, *Chaukhamba Sanskrit Sansthan* Reprint, 2021; 387.
5. Vrddha vagbhata, *Ashtangasangraha with shashilekha commentary of Indu*, translated by Prof. Jyotir Mitra, edited by Dr. Shivaprasad Sharma, *shareerasthana*, chapter Number 1, *putrakameeya adhyaya*, Varanasi, *Chowkhamba Sanskrit series office*, 273.
6. Vagbhata, *Ashtangahrudaya with sarvangasundara commentary of arunadatta and Ayurveda rasayana commentary of Hemadri*, collated by Dr. Anna Moreshwar Kunte and Krishna Ramachandra Shastri Navre, edited by Pt Bhisacharya Harishastri Paradkar Vaidya, *shareerasthana*, chapter Number 1, *Garbhavakranti Shareera*, Varanasi, *Chowkhamba Krishnadas Academy*, 369-370.
7. Vrddhajivaka, *Kashyapa samhita*, revised by Vatya with Sanskrit introduction by Nepal Rajguru Pandit Hemaraja Sharma with Vidyotini Hindi commentary and Hindi translation of Sanskrit Introduction by Ayurvedalankara Sri Satyapaala Bhishagacharya, *Khilasthana*, Chapter Number 10, *Antarvartni Chikitsa Adhyaya*, Bhavan, Varanasi, *Chaukhambha Sanskrit*, 304.

8. Vagbhata, *Ashtangahrudaya with sarvangasundara commentary of arunadatta and Ayurveda rasayana commentory of Hemadri*, collated by Dr. Anna Moreshwar Kunte and Krishna Ramachandra Shastri Navre, edited by Pt Bhisacharya Harishastri Paradkar Vaidya, *shareerasthana*, chapter Number 1, *Garbhavakranti Shareera*, Varanasi, Chowkhamba Krishnadas Academy, 373.
9. Agnivesha, *Charakasamhita*, revised by Charaka and Drudabala, *Ayurveda Deepika of Chakrapanidatta*, edited by Vaidya Jadavji Trikamji Acharya, *Shareerasthana*, Chapter Number 8, *Jatisutriya shareera*, New Delhi, Chaukhambha publications, 346.
10. Sushrutacharya, *Sushruta samhita with Nibandhasangraha commentary of Sri Dalhanacharya and Nyayachandrika Panjika of Gayadasacharya on Nidanasthana*, Edited by Vaidya Jadavaji Trikamji Acharya, *Shareerasthana*, Chapter Number 10 *Garbhini vyakarana Shareera adhyaya*, Varanasi, Chaukamba Sanskrit Sansthan Reprint, 2021; 387.
11. Vagbhata, *Ashtangahrudaya with sarvangasundara commentary of arunadatta and Ayurveda rasayana commentory of Hemadri*, collated by Dr. Anna Moreshwar Kunte and Krishna Ramachandra Shastri Navre, edited by Pt Bhisacharya Harishastri Paradkar Vaidya, *shareerasthana*, chapter Number 1, *Garbhavakranti Shareera*, Varanasi, Chowkhamba Krishnadas Academy, 372.
12. Agnivesha, *Charakasamhita*, revised by Charaka and Drudabala, *Ayurveda Deepika of Chakrapanidatta*, edited by Vaidya Jadavji Trikamji Acharya, *Sutrasthana*, Chapter Number 4, *Shadvirechanashatashriteeya adhyaya*, New Delhi, Chaukhambha publications, 34.
13. Agnivesha, *Charakasamhita*, revised by Charaka and Drudabala, *Ayurveda Deepika of Chakrapanidatta*, edited by Vaidya Jadavji Trikamji Acharya, *Sutrasthana*, Chapter Number 4, *Shadvirechanashatashriteeya adhyaya*, New Delhi, Chaukhambha publications, 32.
14. Agnivesha, *Charakasamhita*, revised by Charaka and Drudabala, *Ayurveda Deepika of Chakrapanidatta*, edited by Vaidya Jadavji Trikamji Acharya, *Sutrasthana*, Chapter Number 25, *yajjapurushheeya adhyaya*, New Delhi, Chaukhambha publications, 132.
15. Agnivesha, *Charakasamhita*, revised by Charaka and Drudabala, *Ayurveda Deepika of Chakrapanidatta*, edited by Vaidya Jadavji Trikamji Acharya, *Shareerasthana*, Chapter Number 4, *Mahati garbhavakranti shareera*, New Delhi, Chaukhambha publications, 320.

TAKE HOME MESSAGE



MOTHERHOOD IS DEVINITY.IT IS HIGHEST, HOLIEST SERVICE TO BE
ASSUMED BY MANKIND

TAKE CARE TO THE OPTIMUM

